

Statement of Deficiency Report

Department of Health
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Sundown M Ranch
2280 State Route 821
Yakima, WA 98901-8302

Agency Name and Address

Mark Loes

Administrator

Investigation

Inspection Type

Tuesday, February 25, 2025

Investigation Start Date

46151

Investigator Number

2025-1825

Case Number

RTF.FS.00001064

License Number

RTF – CD Intensive Inpatient
CD Recovery House
CD Subacute Detoxification

BHA/RTF Agency Services Type

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the investigation.

Deficiency Number and Rule Reference	Findings	Plan of Correction
WAC 246-337-065 – Safety and Security (4)(b) The licensee must protect resident safety and security by developing written policies and procedures that are consistent with the requirements of this chapter and address conducting resident searches. WAC 246-337-075 – Resident rights (2)(h) the licensee must establish a process that ensures resident rights are protected and that each patient has a safe and clean environment.	Based on policy and procedure review, clinical record review, and interviews, the facility failed to protect a patient’s safety and their rights by failing to develop written policies and procedures that addressed a safe environment that included conducting patient searches. Failure to protect a patient’s safety and their rights by not developing policies and procedures that addressed completing resident searches can result in illegal substances being brought into the facility and creating an unsafe environment, causing potential patient harm, including death by overdose. Findings included: 1. A review of the policy and procedure titled “Patient Searches”, undated, showed adult patients may have had their rooms and personal items searched at any time the facility had reason to believe the adult patient possessed contraband. The consent to search was signed by each patient. The search policy applied to the patient served, their belongings, and the physical facility. 2. The Investigator conducted an interview with Staff A, Director of Operations, on 03/19/25 at 9:00 AM. During the interview, Staff A reported that staff observed patients unpack, but the facility does not automatically complete searches of property, body searches, or body checks.	WAC 246-337-065, WAC 246-337-075 Mark Loes Director of Operations is responsible. Patient Search policy and procedure updated August 7, 2025. Policy updated to more clearly describe intake searches of adult patients. Staff trainings that occur weekly and monthly will include training on the updated policy. Discussed with Clinical Leadership and IT to add a check box to the initial health screen to ensure searches are addressed on every adult patient. The evidence that the search occurred will be added to the initial health screen.

	3. A review of the clinical record of Patient #1 showed a document titled "Medical Assessment Initial Interview", dated 01/03/25. The document showed that the patient was admitted to the facility for alcohol use. The patient self-reported that they last used alcohol on 01/03/25 and had been drinking a fifth a day for the last 20 years. The document showed that the patient was intoxicated at the time of admission. The patient had a BAL of .03, their blood pressure was 134/97, and their pulse was 93. The document does not show that a patient search was completed. 4. A review of the clinical record of Patient #1 showed a document titled "Withdrawal Management Initial", dated 01/03/2025. The document showed that the patient's vital signs were taken. The patient's blood pressure was 148/90, pulse rate was 101, oxygen level was 98, and their temperature was 97.2. The patient denied having any suicidal ideation or homicidal ideation. The document did not show that a patient search was completed. 5. A review of the clinical record of Patient #1 showed a document titled, "Incident Report", dated 01/08/25. The document showed that the patient was on 30-minute checks. At 12:30 AM, staff checked on the patient, and they were asleep. At 1:00 AM, staff found the patient unresponsive and had vomited. Staff called 911 and administered Narcan to the patient. Staff also administered Cardiopulmonary Resuscitation (CPR). The document showed that Emergency Medical Services (EMS)	
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	responded and worked on the patient for 15 minutes and got a pulse. The patient was stabilized and transported to the local hospital. A baggie of blue and orange pills was found next to the patient. 6. A review of the toxicology report of patient #1, dated 01/28/25, showed that the patient had postmortem blood testing. The report showed that the patient had a positive finding for fentanyl, Norfentanyl, and TCH (cannabis).	
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WAC 246-337-080 – Resident care services (2)(h)(i)(ii) The licensee must address serious illness, medical emergencies, or threat to life, to include the criteria for determining the degree of medical stability of patients and observe patients for signs and symptoms of illness or trauma.	Based on an email exchange, clinical record review, and a review of the policy and procedures, the facility failed to complete the required amount of the Clinical Institute Withdrawal Assessment (CIWA) for Patient #1. Failure to follow the policy and procedure directing staff on how to complete the required assessments can result in a lack of clinical oversight and poor patient care that may result in serious patient harm, including death. Findings included: 1. A review of the policy and procedure titled “Scope of Services: Withdrawal Management (WDM)”, undated, showed that the facility offered withdrawal management and detox services. The facility ensured the safety of patients served by assessing the patients' needs. The Medical Director or designee oversaw the withdrawal management program. The policy and procedure showed that there were standing orders for alcohol withdrawal. The standing orders included checking the Clinical Institute Withdrawal Assessment for Alcohol (CIWA) and vital signs at admission; every two hours on day one; every three to four hours on day two and three; daily, or when needed while awake, on day four, and daily each day after. A patient with a BAL that exceeded .08 was staffed with management. If the patient's BAL exceeded .35, the patient was screened by medical staff. 2. A review of the clinical record of Patient #1 showed a document titled “Assessments”, dated 01/03/25. The	WAC 246-337-080 Updated WIDGET within the EHR to collect data on WDM patients, so staff have dynamic access to patient data that includes when checks are due and when they are completed. Also provides biometric patient data on each check. For example, a patient in WDM for alcohol withdrawal will appear in the widget and the widget will provide data on when the next check is due, all previous checks and previous biometric data of each check. Staff are regularly trained in widget usage, as well as WDM policies and procedures which includes but is not limited to time frames for WDM checks. Med Room/WDM Trainings occur at least monthly. This will ensure that the policy and procedure regarding time frames for patient checks is followed. Mark Loes Director of Operations is responsible for making the changes. The changes were already implemented after the internal investigation of the incident. May of 2025. Regular audits of the widget will be conducted by the Clinic Coordinator—a min of quarterly. The results of these audits will be provided to the Dir of Operations and the Dir of Programs.
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	document showed that CIWA Scale assessments were completed at 1:50 PM, 6:57 PM, 9:32 PM, and 10:50 PM. The document showed that CIWA Scales were not completed every two hours on day one. 3. A review of the clinical record of Patient #1 showed a document titled "Assessments", dated 01/04/25. The document showed that a CIWA scale was completed at 12:07 AM, 1:57 AM, 8:57 AM, 1:03 PM, 1:43 PM, 4:54 PM, and 5:27 PM. The document showed that the CIWA Scales were not completed every three to four hours on day two. 4. A review of the clinical record of Patient #1 showed a document titled "Assessments", dated 01/05/25. The document showed that a CIWA scale was completed at 8:18 AM, 3:30 PM, 10:38 PM, and 11:59 PM. The document showed that the CIWA Scales were not completed every three to four hours on day three. 5. A review of the clinical record of Patient #1 showed a document titled "Progress Note", dated 01/04/25 at 4:01 AM, 5:55 AM, 7:05 AM, 3:05 PM, and 9:35 PM. The document showed that staff attempted to see the patient for withdrawal management. The patient was asleep for the evaluation. At 4:12 AM, the patient was found unresponsive. 6. A review of the clinical record of Patient #1 showed a document titled "Progress Note", dated 01/04/25 at 9:35 AM. The document showed that the patient was found sedated, and they weren't breathing normally. Staff called Emergency Medical Services (EMS), and they	
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	arrived around 2:50 AM. EMS took the patient's vitals, and the patient refused to be transported to the ER. The patient was placed on thirty-minute checks until they were seen by the medical team the following day. 7. A review of the clinical record of Patient #1 showed documents titled "Progress Note", dated 01/05/25 at 1:25 AM, 4:00 AM, and 7:20 AM. The document showed that staff attempted to see the patient for withdrawal management. The patient was sleeping during the evaluation. 8. In an email exchange with Staff A, Director of Operations, on 04/22/25 at 10:02 AM, Staff A reported they had all the CIWA pulled for Patient #1 and would send them to the Investigator. 9. A review of the clinical record of Patient #1 showed a document titled "Incident Report", dated 01/08/25. The document showed that the patient was on 30-minute checks. At 12:30 AM, the staff checked on the patient, and they were asleep. At 1:00 AM, staff found the patient unresponsive and had vomited. Staff called 911 and administered Narcan to the patient. Staff also administered Cardiopulmonary Resuscitation (CPR). The document showed that EMS responded and worked on the patient for 15 minutes and got a pulse. The patient was stabilized and transported to the local hospital. A baggie of blue and orange pills was found next to the patient. Law enforcement responded to the scene and investigated the incident.	
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WAC 246-337-075 – Resident rights (2)(g) Each resident has the right to have a safe and clean environment.	Based on interviews, clinical record review, and policy and procedure review, the facility failed to establish a process to ensure patients’ rights were protected by failing to create a safe environment and by failing to ensure patient observations were completed. Failure to ensure that patient rights are protected can result in patient harm. Failure to monitor access and egress to the facility can result in a lack of oversight of the facility and poor patient outcomes, including death. Findings included: 1. A review of the policy and procedure titled, “Staff Observation of Persons Served”, undated, showed that the facility ensured the safety of the patient served. The facility provided adequate staff to manage the safety and security of the patient. The facility had adequate staff for each shift for each program. The clinical supervisor managed scheduling, training, duties, and activities of personnel. Clinical personnel were on-site seven days a week from 7 AM to 9 PM. The facility had a medical team that met each morning, Monday through Friday, to conduct pass-down. Each shift manager was required to meet and review the pass-down and discuss patient care. The staff signed a pass-down form to ensure they reviewed the needs of the patient. If a patient was on a special observation, 30-minute checks, or hourly checks, those checks were documented on the patient check form. The patient check form was kept in the administration office to allow all staff access to the information. When the	WAC 246-337-075 Mark Loes Director of Operations is responsible for ensuring correction(s). August 7, 2025, discussed with Clinical Team and Clinic Coordinator. Clinical Supervisors will ensure that checks and documentation of checks meet SMR policies and procedures. Timelines and documentation will be covered weekly. The supervisor who requested the checks is responsible for auditing the documentation to ensure the checks were completed and documented timely and according to policy. The Clinic Coordinator is responsible for auditing WDM checks. Weekly and monthly trainings will include the policy and procedure on both WDM and other requested checks. The clinical supervisors will cover this monthly with support staff, and the Clinic Coordinator will cover WDM checks with the med room staff weekly.
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	patient check form was completed, it was scanned into the patient record. 2. A review of the clinical record of Patient #1 showed a document titled "Hourly Checks", dated 01/03/25. The document showed that the patient was observed between the hours of 4:00 PM and 10:30 PM. The date, time, location of the patient, and staff initials were present on the form. The document showed missing entries from 9:00 PM and 11:00 PM. The document showed that on the same form, the patient was observed on 01/04/25 from 12:00 AM until 4:30 AM. The date, time, location of the patient, and staff initials were present on the form. 3. A review of the clinical record of Patient #1 showed a document titled "Hourly Checks", undated. The document showed the patient was observed on 01/04/25 at 8:00 PM reading a book. The document showed the staff observed the patient sleeping at 9:00 PM through 5:00 AM on 01/05/25. The document showed a missing entry at 10:00 PM and a missing entry from 01/05/25 at 6:00 AM. The patient was observed sleeping again at 7:00 AM. At 8:15 AM, the patient was observed in the front office and observed walking at 9:00 AM. 4. A review of the clinical record of Patient #1 showed a document titled "Hourly Checks", dated 01/05/25. The document showed that the patient observations were completed each hour between 10:00 AM and 1:00 PM. The date, time, location of the patient, and staff initials	
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	were present on the form. The observation form does not have any entries after 1:00 PM, and the next entry occurred on 01/06/25 at 4:30 PM. 5. A review of the clinical record of Patient #1 showed a document titled "Hourly Checks", dated 01/06/25. The document showed observations between the hours of 4:30 PM and 12:30 AM. The date, time, location of the patient, and staff initials were present on the form. Observation entries were missing at 5:00 PM, 5:30 PM, and 6:00 PM. 6. A review of the clinical record of Patient #1 showed a document titled "Hourly Checks", dated 01/07/25. The document showed that patient observations occurred starting at 9:00 AM to 11:00 PM. The document showed that observations were missing from 10:00 AM through 5:00 PM and at 11:00 PM. The document showed that the date, time, location of the patient, and staff initials were present on the form. 7. A review of the clinical record of Patient #1 showed a document titled "Incident Report", dated 01/08/25. The document showed that the patient was on 30-minute checks. At 12:30 AM, staff checked on the patient, and they were asleep. At 1:00 AM, staff found the patient unresponsive and had vomited. Staff called 911 and administered Narcan to the patient. Staff also administered Cardiopulmonary Resuscitation (CPR). The document showed that Emergency Medical Services (EMS) responded and worked on the patient for 15 minutes and got a pulse. The patient was stabilized and	
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Plan of Correction Instructions

Introduction

We require that you submit a plan of correction for each deficiency listed on the statement of deficiency form. Your plan of correction must be Submitted to DOH within fourteen calendar days of receipt of the list of deficiencies.

You are required to respond to the statement of deficiencies by submitting a plan of correction (POC). Be sure to refer to the deficiency number. If you include exhibits, identify them and refer to them as such in your POC.



STATE OF WASHINGTON
DEPARTMENT OF HEALTH

August 20, 2025

Re: Case Number: 2025-1825
License Number: RTF.FS.00001064

Dear Mark Loes:

This letter is to inform you that after careful review of the Plan of Correction (POC) you submitted for the investigation recently conducted at your agency, the Department has determined that the POC is acceptable. You stated in your plan that you will implement corrective actions by the specified timeline. By this, the Department is accepting your Plan of Correction as your confirmation of compliance.

Based on the scope and severity of the deficiencies listed in your statement of deficiency report, the Department will not conduct an unannounced follow-up compliance visit to verify that all deficiencies have been corrected.

The Department reserves the right to pursue enforcement action for any repeat and/or uncorrected deficiencies based on applicable statute and rules.

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